An Open Letter to Healthcare Professionals

I'm a scientist by training, though not in clinical medicine. My work revolves around **research, mechanistic modeling, and publishing in peer-reviewed journals**. Because of my background, I'm comfortable with **mechanistic extrapolation:** understanding how a biological system works, imagining a change, and predicting how the system will respond. When you're careful and thorough, **you're often right**.

But that's not how the FDA approves drugs.

Regulatory approval requires statistically demonstrated safety and efficacy across large, expensive, multi-phase clinical trials. If a drug doesn't promise enough profit, the trials don't get run, even if the compound is plausibly effective and safe. If the intellectual property doesn't support exclusivity, the trials don't get run. And if a compound improves **healthspan or physiological resilience** rather than treating a specific disease, the trials don't get run either, because FDA endpoints must be disease-specific. **"Making healthy people healthier" is not an approvable indication.**

This leaves us with a growing category of **peptides, small molecules,** and **biologics** supported by strong mechanistic evidence, promising preclinical data, and often early human signals, but **no Phase III trials**. That reality leads people like me to conduct **careful, informed N=1 research** on ourselves.

I take that responsibility seriously. This is my version of informed consent: before I inject anything subcutaneously, I do a **deep dive into the published literature** of in vitro studies, animal models, early human trials, and mechanistic reviews. I look for **short-term safety signals**; if it's not there, it's a no-go. The lack of PK data is always a challenge, so dosing requires **caution, conservatism, and iteration**. But with enough research, you can build a reasonable picture of how a peptide behaves biologically.

Here's what that looks like in my stack:

- **Retatrutide for metabolic stability** — my CGM data shows profound improvements. It should be inexpensive and widely available to everyone near prediabetic ranges.
- **Tesamorelin + Ipamorelin every morning, and Ipamorelin + CJC-1295 NODAC every evening**, cycled 12-weeks ON and 6-weeks OFF to prevent receptor desensitation and pathway adaptation. They support **body recomposition, reduce visceral fat, improved sleep, and enhanced tissue repair**. My labs reflect these changes, and I feel the functional difference , and I miss those benefits during off-cycles.

- **Thymulin and Thymosin α1 twice weekly** for immune modulation. It's hard to "feel" immune improvement, but **Thymulin resolved a chronic overnight post-nasal drip** I'd had for years.
- **GHK-Cu daily** for tissue repair, and **KPV daily** for inflammaging.
- **TB-500 and BPC-157**, cycled 2 weeks on / 2 weeks off, produce **noticeable recovery benefits** after hard workouts or heavy yardwork.
- **ARA-290 in a staggered weekly pattern (4/2/1 mg M/W/F)** for **small-fiber nerve protection, microvascular resilience, and systemic inflammatory modulation**.
- And although they're not peptides, I also take **metformin and tadalafil off-label** because the **peer-reviewed healthspan literature is extraordinary**. The real question isn't why I'm using them, it's why these inexpensive, well-studied treatments **aren't standard of care for aging populations**.

Some peptides carry real risks, for example **MT-II has clear cancer concerns.** Some peptides, like **GH secretagogues require monitoring.** Other peptides, like **TB-500 are strongly angiogenic and warrant cancer-screening conversations.** Others have **excellent safety profiles but mixed efficacy**, like DSIP. And some, like SS-31, **remain popular despite failing Phase III endpoints**.

Some peptide users are **thoughtful, careful, and research-driven**. Others are inexperienced, relying on gym-bro advice or Reddit threads about how to reconstitute lyophilized powder or what to "stack."

Some of your patients are already using BPC-157, TB-500, GHK-Cu, KPV, or Retatrutide. And more will be using them soon. You and your patients need an **informed partnership**. The labs, diagnostics, and treatments you choose need to take into consideration what they have chosen to do.

That requires them to feel **safe telling you everything they're doing**. And they won't tell you if they expect to be lectured. It's perfectly appropriate to say: **"These are not FDA-approved for human use. I cannot recommend them. We don't have sufficient clinical data to know they're safe or effective."** But it's equally important to add: **"Thank you for telling me. I'm not judging you. I want to take this into account as I care for you."**

That simple acknowledgment opens the door to **safer, more informed care**.

And while you don't need to become peptide experts, **some awareness makes you better clinicians**. You don't need hours to catch up; you can ask an AI research assistant to summarize the clinical and preclinical literature on BPC-157, TB-500, GHK-Cu, KPV, or any other peptide. **You'll be surprised how quickly you can get oriented.**

If a patient is using **GH secretagogues like Tesamorelin, Ipamorelin, or CJC-1295**, you'll want **IGF-1** in their annual labs. If they're using **angiogenic peptides, like TB-500**, you'll want to discuss **cancer risk and screening**. If they mention **"Glow blend,"** you'll need to know it contains **TB-500**.

I have a model relationship with my PCP. I inform her about everything. **All my peptides are logged in my EHR.** We discuss what additional labs are appropriate. She never endorses my choices, but she doesn't scold me either.

My hope is that healthcare professionals stay **informed enough to help patients navigate these choices safely, compassionately, and without judgment**. The landscape is changing. Patients are experimenting. Some are doing it well; some are not. But the **mechanistic, preclinical, and early clinical evidence** strongly suggests that certain peptides may offer **meaningful health benefits when used responsibly**.

Thank you for taking the time to understand this emerging space. Your willingness to stay informed makes a profound difference in patient safety and trust.